SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

A. Patient Information

Case type	☐ Initial case ☑ Follow-up case
1. Patient initials	ABC
2. Age / date of birth	68 years; XX/XX/19XX
3. Gender	☐ Male ☑ Female ☐ Other
4. Weight	58 kg

B. Suspected Adverse Reaction

5. Reaction start date	Day 3 after clarithromycin initiation; XX/XX/20XX. Mild thigh and calf pain began.
6. Reaction stop date	Day 14 (clinical recovery reported); XX/XX/20XX
7. Event / reaction description and management	After clarithromycin 500 mg twice daily was added to long-term simvastatin 40 mg at bedtime, the patient developed thigh and calf pain on Day 3. On Day 4 she developed marked proximal muscle weakness, difficulty climbing stairs and cola-coloured urine. She was admitted on Day 6. Findings included CK 18,500 U/L, creatinine 1.9 mg/dL, potassium 5.6 mmol/L and positive urine myoglobin, consistent with rhabdomyolysis with acute kidney injury and hyperkalemia. Clarithromycin and simvastatin were stopped on Day 6. IV fluids and electrolyte monitoring were provided from Day 6–10. CK trended down, urine cleared, and by Day 14 creatinine was 1.0 mg/dL and weakness had resolved.
12. Relevant investigations with dates	Day 6: CK 18,500 U/L (reference <170); creatinine 1.9 mg/dL (0.5–1.1); potassium 5.6 mmol/L (3.5–5.0); AST 420 U/L (reference <40); ALT 210 U/L (reference <40); urine myoglobin positive; TSH normal. Day 10: CK trending down (exact value not provided). Day 14: creatinine 1.0 mg/dL.
13. Relevant medical / medication history	Hypertension and dyslipidemia. Baseline creatinine 0.9 mg/dL. No muscle complaints before this event. Regular medicines: simvastatin 40 mg HS for 3 years; amlodipine 5 mg OD for 3 years; hydrochlorothiazide 12.5 mg OD for 2 years. Clarithromycin 500 mg BD newly prescribed for lower respiratory tract infection. Allergy history not provided. Trauma, strenuous exercise, alcohol use and seizures excluded. TSH normal; hypothyroidism excluded.
14. Seriousness	☐ No ☑ Yes ☐ Death ☐ Congenital anomaly ☐ Life-

	threatening ☐ Disability ☑ Initial hospitalization ☑ Other medically important (rhabdomyolysis with acute kidney injury and hyperkalemia)
15. Outcome	☑ Recovered ☐ Recovering ☐ Not recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown Clinical recovery by Day 14; weakness resolved and creatinine returned to 1.0 mg/dL.

C. Suspected Medication(s)

8. Suspected medicines. Manufacturer, batch/lot and expiry details were not supplied in the case.

No.	Drug (generic)	Manufacturer	Batch / lot	Expiry	Dose	Route	Freq.	Started	Stopped	Indication
i	Clarithromycin	abc	xyz	xyz	500 mg	oral	BD	Day 0	Day 6 (stopped early)	Lower respiratory tract infection
ii	Simvastatin	abc	xyz	xyz	40 mg	oral	HS	3 years before event; exact date unknown	Day 6	Dyslipidemia

9–10. Action taken after reaction and rechallenge

Suspected drug	Withdrawn	Dose increased	Dose reduced	Unchanged	N/A	Unknown	Reappeared?	Dose if reintroduced
Clarithromycin	☑	☐	☐	☐	☐	☐	☐ Yes ☑ No ☐ Unknown	Not applicable
Simvastatin	☑	☐	☐	☐	☐	☐	☐ Yes ☑ No ☐ Unknown	Not applicable

11. Concomitant medical products (excluding medicines used to treat the reaction)

Medicine	Dose	Route	Frequency	Date started	Date stopped	Indication
Amlodipine	5 mg	oral	OD	3 years before event; exact date unknown	Ongoing at Day 0; stop date not provided	Hypertension

Hydrochlorothiazide	12.5 mg	oral	OD	2 years before event; exact date unknown	Ongoing at Day 0; stop date not provided	Hypertension

D. Reporter Details

16. Name and address	Abc ; xxxxxxxxx
PIN / email / contact number	70xxxxxxxx
Occupation	abc
Signature	abc
17. Date of this report	xx/xx/20xx
Signature and name of receiving personnel	abc